

Clinic Note

John Smith

Age 55 - Male

MRN N/A

Date of Birth 01/18/1970

Date of Service 04/28/2025

History of Present Illness

Mr. John Smith, a 55-year-old male with a past medical history of hypertension and hyperlipidemia, presents to the emergency department with a chief complaint of chest pain that started 2 hours ago.

He describes the pain as a pressure-like sensation located in the center of his chest, radiating to his left arm. The pain began while he was mowing the lawn and has persisted despite resting. He rates the pain as 7/10 in severity.

He reports associated symptoms of shortness of breath, nausea, and mild diaphoresis. He denies any syncope, palpitations, or recent trauma.

He has no prior history of similar episodes. He denies recent travel, leg swelling, or calf pain.

Review of Systems

General: Positive for diaphoresis, negative for fever or weight loss.

Cardiovascular: Positive for chest pain and shortness of breath; negative for palpitations or edema.

Respiratory: Mild shortness of breath; no cough or wheezing.

Gastrointestinal: Positive for nausea; denies vomiting, diarrhea, or abdominal pain.

Neurological: Denies dizziness, syncope, or focal weakness.

Musculoskeletal: No chest wall tenderness or recent injuries.

Physical Exam

General: Alert and oriented x3, appears mildly distressed due to pain.

Vital Signs: BP 148/92 mmHg, HR 102 bpm, RR 20/min, Temp 98.4°F, O2 Sat 96% on room air.

Cardiovascular: Regular rhythm, tachycardic, no murmurs, rubs, or gallops. No jugular venous distension (JVD).

Respiratory: Clear to auscultation bilaterally, no rales or wheezing.

Gastrointestinal: Soft, non-tender, non-distended abdomen.

Musculoskeletal: Chest wall non-tender to palpation.

Extremities: No edema, no calf tenderness.

Neurological: No focal deficits.

Assessment and Plan

Assessment:

Chest Pain - Likely acute coronary syndrome (ACS).

Hypertension.

Hyperlipidemia.

Plan:

Immediate Actions:

Administer aspirin 325 mg chewed.

Start oxygen if saturation <94%.

Obtain 12-lead EKG (already pending).

Place patient on cardiac monitor and establish IV access.

Obtain cardiac enzymes (troponin I), complete blood count (CBC), basic metabolic panel (BMP), and chest X-ray.

Medications:

Nitroglycerin 0.4 mg SL q5 minutes x3 doses as needed for pain, monitor BP.

Consider morphine if pain persists despite nitroglycerin.

Further Management:

Consult cardiology for possible admission and further evaluation (possible cardiac catheterization).

Consider heparin drip depending on EKG and troponin results.

Monitoring:

Continuous telemetry monitoring.

Repeat troponins in 3 and 6 hours.

Patient Instructions

If discharged:

Seek immediate medical attention for recurrent chest pain, shortness of breath, palpitations, or fainting.

Take medications exactly as prescribed.

Avoid strenuous activities until cleared by cardiology.

Follow up with primary care provider and cardiology within 1 week.

Lifestyle modifications: low-salt, low-fat diet, regular exercise as tolerated, and smoking cessation if applicable.

X

Mr. Chandan Ravishankar, Dev

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